Medical confidentiality

Ethical foundations, Bulgarian law, the limits of disclosure, European case law and liability — with ten cases

Practical Class 3 · Social Medicine and Medical Ethics I · 2026/2027

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Plan of the class

Block	Cases
1 · Concept and ethical foundations	—
2 · The Bulgarian legal framework	1–2 (plenary)
3 · Disclosure: consent, law, danger to others	3–6 (plenary)
4 · The European Court of Human Rights	—
5 · Breaches, digital care, liability	7–10 (small groups)

How we work

- each case: facts → questions → your decision → **key points**
- always ask: **what information, who asks, on what basis, how much, through which channel — and is it recorded?**
- Bulgarian law is the reference; foreign decisions are marked *comparative*; quotations of Bulgarian acts are working English translations

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2. The Bulgarian legal framework
3. Disclosure: consent, law and danger to others
4. The European Court of Human Rights
5. Breaches, digital care and legal liability

Four related but distinct concepts

Concept	Meaning	Typical breach
Privacy	the person's control over access to body, personal space, relationships, communications and information	unnecessary observers during an examination
Confidentiality	the duty of a person or institution that receives private information in a relationship of trust not to disclose or use it beyond its purpose	telling a relative the diagnosis without authorisation
Medical confidentiality	the physician's professional duty to keep what is learned in practice — the classic core of the duty (Code of Professional Ethics, Arts 51–55)	discussing a recognisable patient in the cafeteria
Data protection	the legal and organisational rules for the whole life cycle of data: collection, access, sharing, storage, deletion	shared passwords; records opened without a care relationship

Confidentiality is the patient's right and the professional's duty — not the doctor's privilege.

Why we keep confidences: four justifications

Consequences

patients who fear disclosure conceal facts, delay testing, avoid care — harm to them and, with infections, to others

Autonomy and rights

the person decides who learns intimate facts about him; private life is protected by the Constitution and the ECHR

Fidelity

the patient becomes vulnerable for one purpose — care; the professional implicitly promises not to use the information otherwise

Virtue

discretion, respect and trustworthiness are part of the character of a good physician

A modern problem: confidentiality in teams

Siegler counted about **75 people** with a legitimate reason to see one hospital record and called traditional confidentiality "a decrepit concept". The conclusion is not that confidentiality is obsolete, but that in modern care it is protected by **access rules, audit trails and purpose limitation** — not only by the discretion of one doctor.

Siegler M. Confidentiality in medicine — a decrepit concept. N Engl J Med 1982;307:1518–1521. doi:10.1056/NEJM198212093072411

From the Hippocratic Oath to the Declaration of Geneva

> *What I may see or hear in the course of the treatment or even outside of the treatment in regard to the life of men, which on no account one must spread abroad, I will keep to myself, holding such things shameful to be spoken about.*
>
> — Hippocratic Oath (transl. L. Edelstein)

> *I will respect the secrets that are confided in me, even after the patient has died.*
>
> — WMA Declaration of Geneva (2017)

> *(2) Medical confidentiality is kept also with regard to the members of the patient's family. (3) The physician keeps the secrets entrusted by the patient also after his death.*
>
> — Code of Professional Ethics for Physicians in Bulgaria, Art. 51(2)–(3)

What changed

- from a **personal promise** of the doctor to a **right** of the patient
- from one doctor to **teams, laboratories, registries and electronic records**
- from "never tell" to **lawful, limited and documented** sharing

What is protected

> *Health information is personal data related to the state of health and the physical and mental development of persons, as well as any other information contained in medical prescriptions, orders, protocols, certificates and other medical documentation.*
>
> — Health Act, Art. 27(1)

Also protected: the **fact** that a person sought care, appointments, images, recordings, samples, genetic results, family and social facts learned during care, and the professional's opinions.

Data	Status
Identifiable	name, personal number, face, address — personal data
Indirectly identifiable	rare diagnosis + age + town + date; a tattoo; a wristband — still personal data
Pseudonymised	a code replaces the name, but a key restores the link — **still personal data**
Anonymised	cannot reasonably be linked back to a person — outside data-protection law

Removing the name does not make information anonymous.

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Levels of regulation

Level	Main content
Constitution of the Republic of Bulgaria, Art. 32(1)	private life is inviolable; protection against unlawful interference in private and family life
ECHR, Art. 8; Oviedo Convention, Art. 10(1)	respect for private life, including information about one's health
Regulation (EU) 2016/679 (GDPR) and the Personal Data Protection Act	health data are a **special category** (Art. 9); breach notification; complaints and damages
Health Act, Arts 27–28d	health information, grounds for disclosure, access by the patient and heirs, prohibition on disclosure, the national health information system
Code of Professional Ethics for Physicians, Arts 51–55	content and scope of medical confidentiality; team exemption; teaching; statutory reporting
Penal Code, Labour Code, Obligations and Contracts Act, Professional Organisations Act	criminal, disciplinary, civil and professional consequences of a breach
Special acts (e.g. Child Protection Act, Art. 7)	duties to report that override professional secrecy

Code of Professional Ethics for Physicians, Section IV

Medical confidentiality includes all information that the patient has shared with the physician in connection with his condition, the facts found during the examination and tests, as well as everything the physician has learned about the patient in practising his profession.

— Art. 51(1)

Medical confidentiality extends to all medical documentation and illustrative material, as well as to the data and conclusions of consultations.

— Art. 52

When several physicians treat the same patient simultaneously or successively, they are free from medical confidentiality among themselves, except where the patient has a reasoned objection.

— Art. 53

For research and teaching, data and illustrations may be communicated if they guarantee the patient's anonymity. Identification of patients by third parties must not be allowed.

— Art. 54 (abridged)

The physician must inform the relevant institutions in the cases set by law; in these cases he is released from the duty of confidentiality.

— Art. 55 (abridged)

Code of Professional Ethics for Physicians in Bulgaria, ДВ 79/2000, amended ДВ 85/2013; text checked in APIS 12.11.2026

The statutory prohibition and the duty to protect

> *Medical specialists and employees of medical establishments have no right to disclose information about the patient obtained in the performance of their official duties.*
>
> — Health Act, Art. 28c

> *The persons under Art. 27(2) are obliged to ensure protection of the health information they store against unauthorised access.*
>
> — Health Act, Art. 28(3)

> *As a patient, everyone has the right to protection of the data relating to his state of health.*
>
> — Health Act, Art. 86(1)(5)

Who is bound

- physicians, dentists, nurses, midwives, pharmacists, laboratory staff
- **all employees** of the establishment: registrars, IT staff, administrators
- students and trainees act under the same rules

Access follows the care relationship

Working in the same hospital gives no right to open a record. Curiosity about a colleague, neighbour or public figure is not a clinical need.

Health Act, Art. 28(1): disclosure to third parties

No.	Health information may be provided to third parties when…	Typical recipient
1	treatment continues in another medical establishment	the receiving team
2	**there is a threat to the health or life of other persons** — after the patient is notified (para. 2)	the person at risk or the competent authority
3	it is necessary to identify a human body or establish the cause of death	forensic medicine, investigating bodies
4	it is necessary for state health control to prevent epidemics and the spread of communicable diseases	Regional Health Inspectorate
5	it is necessary for medical expertise and social insurance	expert medical commissions, National Social Security Institute
6	it is necessary for medical statistics or research, **after identifying data are erased**	statisticians, researchers
7	it is necessary for the Ministry of Health, the National Centre for Health Information, the NHIF, the Regional Health Inspectorates and the National Statistical Institute	named public bodies
8	it is necessary for a licensed insurer (health, accident or life insurance)	the insurer — within the contract; **never human-genome data** (Art. 141(4))

Disclosure needs a lawful basis: a ground in Art. 28(1), the patient's authorisation, a representative's authority or another specific legal provision. Relatives, employers and the media as such have none of these.

The patient, an authorised person and the heirs

> *(1) The patient has the right to receive from the medical establishment the health information concerning his state of health, including copies of his medical documents. (2) The patient has the right to authorise* ***in writing*** *another person to see his medical documents and to make copies of them. (3) On the death of the patient his* ***heirs and relatives in direct and collateral line up to the fourth degree inclusive*** *have the right to see the health information about the deceased and to make copies of his medical documents.*
>
> — Health Act, Art. 28b

Who asks	Basis
the patient	Art. 28b(1); Art. 86(1)(13)
spouse, child, parent of a capable adult	only with a **written authorisation** (Art. 28b(2); Art. 92(4))
heirs after death	Art. 28b(3) — after verifying status

Code and statute after death

The Code keeps secrets "after death" (Art. 51(3)); the Health Act gives heirs access. The **statute prevails**; the Code governs the **manner**: verification, procedure, no more than requested.

EU data protection: the GDPR and the Personal Data Protection Act

Regulation (EU) 2016/679

- **Arts 6 and 9:** processing needs a lawful basis (Art. 6) **and**, for health and genetic data — a special category — an exception to the prohibition in Art. 9(1).
- **Art. 9(2)(h), 9(3):** the exception for diagnosis and treatment by professionals bound by professional secrecy; it does not replace consent **to treatment**.
- **Art. 33:** the controller notifies the supervisory authority without undue delay and, where feasible, **within 72 hours of becoming aware** — unless the breach is unlikely to result in a risk; every breach is documented.
- **Art. 34:** the patient is informed without undue delay when the breach is likely to result in a **high risk**.
- **Arts 82–83:** damages and administrative fines — controllers and, where applicable, processors.

> *Where his rights under Regulation (EU) 2016/679 and this Act are violated, the data subject may refer the matter to the Commission within six months of learning of the violation, but not later than two years after it was committed.*
>
> — Personal Data Protection Act, Art. 38(1)

Art. 39(1)–(2): the data subject may also challenge the controller's actions in court and claim damages for unlawful processing.

Both regimes apply together: the GDPR and the Personal Data Protection Act, and the Health Act and the Code — a disclosure must satisfy all of them.

Case 1 · The waiting spouse

The case

Mr P., 58, is admitted with abdominal pain. Imaging suggests pancreatic cancer; the biopsy is pending. He is alert and has asked the consultant not to discuss the possible diagnosis with anyone until he has spoken to his adult children. His wife arrives while he is having another test, shows her identity card, and asks the ward doctor whether the scan showed cancer: "We have been married for thirty years — I need to prepare the family."

1. Does marriage give her a right to the information?
2. How can the doctor answer honestly without confirming or hinting at the diagnosis?
3. What should be done when Mr P. returns, and how is it recorded?

Case 1 · The waiting spouse — key points

Law

- Kinship gives **no** right to information about an adult with decision-making capacity. Routes the patient controls: written authorisation to see his documents (Health Act, Art. 28b(2)); a person to be informed **instead of** him (Art. 92(4)); or simply his consent to a conversation.
- None of the grounds in Art. 28(1) applies; disclosure would breach Art. 28c and Code Art. 51(2) ("also towards the family").
- Mr P. has expressly limited disclosure — his wish is documented and respected.

Ethics and practice

- The doctor may **listen** and receive information from her without disclosing anything in return.
- Honest wording: "I understand how worried you are. I cannot discuss your husband's results without him; I will tell him you asked, and I can be there when he talks to you."
- Avoid indirect disclosure: silence, facial expression, "it's not what you fear".
- When he returns: tell him, ask whom he wants involved, record any authorisation and its scope.

Case 2 · The phone call on the ward

The case

The ward telephone rings. The nurse answers. The caller asks how a particular patient is doing. Because the nurse knows the patient, she tells the caller that the patient has pneumonia on top of chronic bronchitis and that intravenous antibiotics have been started. When she puts the phone down, you ask who called. She does not know.

1. Was this a breach? Which rule was violated?
2. What should the nurse have done?
3. What do you do now?

Case 2 · The phone call on the ward — key points

Law

- A breach of Health Act Art. 28c (binding on all staff) and of Art. 28(3) (protection against unauthorised access); identity and authority were never checked.
- The establishment, as data controller, documents and assesses the incident; unless it is unlikely to result in a risk, the Commission is notified within 72 hours of becoming aware (GDPR Art. 33) and, if high risk, the patient (Art. 34).

Practice

- Telephone rule: confirm identity **and** the patient's authorisation; call back on a known number; give the minimum.
- Even confirming that someone is admitted discloses information.
- Now: tell the nurse calmly, report through the internal channel, inform the patient honestly, review the ward procedure — no blame culture, no concealment.

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A disclosure decision in six steps

	Step	Questions
1	Information and purpose	What exactly is known? Who asks? Which decision would disclosure support?
2	Consent and authority	What has the patient authorised? Does a ground in Health Act Art. 28(1) or a special act **require or permit** disclosure?
3	The threatened harm	How serious, probable, imminent and preventable? Who is at risk — an identifiable person?
4	Less intrusive options	Persuade the patient, help him disclose, reduce the risk practically, seek anonymised advice.
5	Minimum disclosure	The authorised recipient, the minimum content, a secure channel, the right moment.
6	Consult, inform, record	Senior and legal advice; for danger to others the patient is notified **before** disclosure (Art. 28(2)); document the reasoning.

Never promise absolute secrecy before you know what the patient is going to tell you.

Danger to others: Bulgarian law and comparative decisions

> *Health information may be provided to third parties when there is a threat to the health or life of other persons. ... Information in the cases under para. 1, item 2 is provided after the person concerned has been notified.*
>
> — Health Act, Art. 28(1)(2) and (2)

Clinical and proportionality assessment (not statutory wording)

serious harm · real and probable · identifiable person or group · disclosure can prevent it · less intrusive means considered (without delaying protection) · minimum content to the right recipient

Decision (comparative)	Principle
Tarasoff v Regents of the University of California (California, 1976)	a therapist whose patient presents a serious danger of violence to an identifiable victim must take reasonable steps to protect that person: "the protective privilege ends where the public peril begins"
W v Egdell [1990] Ch 359 (England and Wales)	disclosure of a psychiatric report to the responsible authorities was justified by a real risk of serious harm to the public

Comparative decisions are not Bulgarian law; they show how the same test is reasoned.

Duties to report

> *(1) A person who learns that a child needs protection must immediately notify the Social Assistance Directorate, the State Agency for Child Protection or the Ministry of the Interior. (2) The same obligation applies to any person who has learned this in connection with his profession or activity,* ***even if bound by professional secrecy****.*
>
> — Child Protection Act, Art. 7(1)–(2)

> *... In these cases the physician is released from responsibility for keeping medical confidentiality.*
>
> — Code of Professional Ethics, Art. 55

Mandatory reporting	**Basis and recipient**
child in need of protection	Child Protection Act, Art. 7 — Social Assistance, the Agency or the police
communicable diseases	Health Act, Art. 60(1); Ordinance No. 21/2005 — registration and notification to the Regional Health Inspectorate
HIV	Ordinance No. 47/2009 — coded reporting; disclosure to third parties only with written consent (Art. 30a)
Permitted disclosure	Health Act, Art. 28(1)
identification, cause of death; medical expert assessment	items 3 and 5 — when necessary for that purpose

Reporting is to the designated recipient, to the extent required — not to everyone.

Children, adolescents and adults who cannot decide

Patient	Who consents (Health Act, Art. 87)	What follows for confidentiality
child under 14	the parent or guardian (Art. 87(4))	the parent receives the information needed to decide; the child is informed in an age-appropriate way
minor aged 14–17	the minor **and** a parent or curator (Art. 87(2))	the parent receives what the joint consent requires; other confidences are discussed first with the minor
aged 16–17: the counselling, preventive examinations and tests **listed in Ordinance No. 8/2018** (Art. 87(3))	the minor alone	a strong expectation of confidentiality; diagnosis and treatment beyond the list need joint consent
adult under full interdiction / restricted interdiction; mental disorder with established inability (Art. 87(7), 162(3))	guardian (Art. 87(4)); curator together with the patient (Art. 87(2)); the person appointed under Art. 162(3)	the representative receives what the decision requires; relatives are **not** automatically decision-makers

Limits for every age

A child in need of protection is reported (Child Protection Act, Art. 7). Never promise an adolescent absolute secrecy; explain the limits before the conversation.

Genetic information: a result shared by a family

Two views

- **Individual:** the result belongs to the tested person; he decides.
- **Familial:** it describes a biological relationship; relatives may have a vital interest in it.

Good practice

- pre-test counselling: discuss family implications **before** testing
- support voluntary disclosure (letters for relatives, joint consultation)
- weigh penetrance, severity, preventability, urgency

Law

- Health Act Art. 141(4): human-genome data are personal data and **may not be provided to employers, health-insurance organisations or insurance companies**; Art. 141(3): no discrimination on the basis of genetic results.
- No express rule authorises disclosure **to relatives** against the patient's wish; it would have to meet Art. 28(1)(2) — a threat to the health or life of others — with prior notification (Art. 28(2)). Its application is uncertain and fact-dependent: seek genetic and legal advice.
- *Comparative:* **ABC v St George's** [2020] EWHC 455 (QB) — a duty to balance (and act on the balance) arose from the close relationship between **that** relative and **that** team; no breach established.

The question is not "whose result is it?" but "what may be done to prevent serious harm with the least intrusion?"

Case 3 · The partner does not know

The case

Mr J., 35, consults his general practitioner after unprotected sexual contacts. After counselling, tests are ordered; the HIV test is positive. The doctor offers a joint meeting with his wife to help with disclosure, but Mr J. refuses: his wife must not know. He says he will "be careful". His wife is registered with the same practice.

1. Which steps come before any disclosure?
2. Which legal rules apply to HIV information — and what may be told to the wife?
3. Does it matter that the wife is also the doctor's patient?

Case 3 · The partner does not know — key points

Law

- Special rule: HIV-related health information may not be disclosed to third parties **without the person's express written consent** (Ordinance No. 47/2009, Art. 30a).
- To prevent infection, medical specialists **may inform contacts that they have been at risk** — keeping the identity of the HIV-positive person secret, communicating confidentially and safely, and referring them for counselling and testing (Art. 30b).
- Coded reporting follows the Ordinance and is not a warning to the partner. The wife being a patient of the practice creates a duty of care to her, not a right to his data.

Ethics and practice

- First: counselling, start of treatment, an offer of assisted partner notification; assess the risk of violence after disclosure.
- The risk to her depends on exposure and on his treatment: a sustained undetectable viral load prevents sexual transmission — but past exposure may still need testing.
- If he refuses: notification of the wife under Art. 30b, without naming him as the source (in a couple, she may still infer it — discuss this honestly with him).
- Not legitimate recipients: employer, relatives, friends.

Case 4 · The bus driver with seizures

The case

Ms A., 32, drives an intercity bus. Despite treatment she continues to have seizures. Her neurologist tells her to stop driving passengers immediately while the medical fitness procedure is clarified. Two weeks later she admits that she is still driving full routes — she is the household's only earner — and asks the neurologist to keep it confidential.

1. How serious, probable and preventable is the harm, and to whom?
2. What can be done without disclosure?
3. If disclosure becomes justified, who receives what?

Case 4 · The bus driver with seizures — key points

Law

- Passengers and road users are exposed to a serious, real risk: Health Act Art. 28(1)(2) can apply; she is told before disclosure (Art. 28(2)).
- The recipient is the body able to stop the risk through the procedure for drivers' medical fitness (e.g. the employer's occupational-health service, the medical commission) — not colleagues or the public.
- Content: unfit to drive passengers pending assessment; **not** the full diagnosis or history.

Ethics and practice

- She has already ignored the advice for two weeks: act **now** to stop further passenger driving — no new waiting period. Tell her what you will do and why.
- Offer support (sick leave, social services, a certificate for other duties).
- Her income is a real hardship — it changes **how** we help, not whether passengers may be put at risk.
- Record the conversation, the advice given and the decision.

Case 5 · What do I tell the mother?

The case

K., 16, is operated on at night for suspected appendicitis; her mother consented with her. In theatre the appendix is normal, but there is a bleeding ectopic pregnancy; a gynaecologist removes the right fallopian tube. K. had denied sexual activity in front of her mother. After surgery the surgeon goes to the mother in the waiting room.

1. What does the mother need to know, and what must K. hear first?
2. What does Health Act Art. 87(2) mean for information?
3. How can a lie be avoided without breaking K.'s confidence?

Case 5 · What do I tell the mother? — key points

Law

- Consent covered appendicectomy, not salpingectomy. A different operation without written consent is lawful only when life is **directly threatened** and consent cannot be obtained in time (Health Act, Art. 89(2)) — a bleeding ectopic pregnancy under anaesthesia can meet this; document why.
- At 16 the minor **and** a parent consent (Art. 87(2)); the information under Art. 88 includes the **diagnosis**. The mother, as co-consenting parent for the further care, is entitled to the truthful diagnosis that this care requires. Art. 87(3) covers only the activities listed in Ordinance No. 8/2018.

Ethics and practice

- Now: tell the mother that K. is stable, that the appendix was normal and that bleeding required an urgent operation on the right tube; say that you will explain everything to K. first, as soon as she is awake.
- Talk to K. alone promptly: the diagnosis (ectopic pregnancy), its implications, who will tell her mother and how; a safety check (age of the partner, coercion → Child Protection Act, Art. 7).
- Then the mother receives the diagnosis needed for further care — not unrelated sexual details. Never fabricate a diagnosis.

Case 6 · The records of a deceased mother

The case

A woman died of ovarian cancer less than six months after diagnosis. She had consulted her general practitioner twice with vague symptoms before being referred for ultrasound. Her daughter writes to the practice asking for a full copy of the medical records: the family wants to know whether the cancer could have been diagnosed earlier. You are asked to reply.

1. Does confidentiality continue after death?
2. Is the daughter entitled to the records?
3. How should the practice respond to a possible complaint?

Case 6 · The records of a deceased mother — key points

Law

- Yes: heirs and relatives up to the fourth degree may see the health information and make copies (Health Act, Art. 28b(3)) — a daughter qualifies.
- Verify identity and kinship (e.g. a certificate of heirs) and follow the establishment's procedure.
- The Code's duty after death (Art. 51(3)) does not defeat the statutory right; it governs the respectful manner.

Ethics and practice

- Do not treat the request as hostile; offer a meeting to explain the course of events.
- Preserve the original record; never backdate or conceal changes — any correction or addendum is dated, attributed and traceable.
- A suspected missed diagnosis is a matter for honest review (and, if the family chooses, for a complaint or claim — Class 4); confidentiality is not a shield for errors.

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European Court of Human Rights (1): Article 8 and medical data

Judgment	Facts and principle	Outcome
Z v Finland, no. 22009/93, 25.02.1997	in her husband's criminal trial, her doctors were compelled to testify and records were seized — **justified**; but her identity and HIV status were disclosed in the published judgment and the records were to become public after only ten years — **not justified**. "The protection of personal data, not least medical data, is of fundamental importance" to private life and to trust in the medical profession.	violation (publication; short confidentiality period)
I v Finland, no. 20511/03, 17.07.2008	a nurse with HIV; colleagues in her own hospital could read her record; the system kept no log identifying individual users. The State must ensure **practical and effective** protection — access restrictions **and** individual access logs.	violation of Art. 8
M.S. v Sweden, no. 20837/92, 27.08.1997	a clinic sent the applicant's records to the Social Insurance Office examining her claim for compensation; the transfer pursued a legitimate aim, was relevant and had safeguards.	no violation

Article 8 requires both restraint and organisation: limited purposes and audit trails.

European Court of Human Rights (2): disclosure and presence

Judgment	**What happened — all: violation of Art. 8**
L.L. v France, no. 7508/02, 10.10.2006	a medical report about the husband was used and quoted in divorce proceedings although it was not necessary to decide the case
Biriuk v Lithuania, no. 23373/03, 25.11.2008	a newspaper published that the applicant had HIV, citing hospital doctors; the damages available were inadequate
Avilkina and Others v Russia, no. 1585/09, 06.06.2013	hospitals sent medical records of patients who had refused transfusions to a prosecutor — no pressing social need, no adequate safeguards
Konovalova v Russia, no. 37873/04, 09.10.2014	medical students were present at a birth without adequate information and consent
Surikov v Ukraine, no. 42788/06, 26.01.2017	an employer collected and relied on decades-old psychiatric information in a promotion decision
Mockutė v Lithuania, no. 66490/09, 27.02.2018	a psychiatric hospital disclosed information about the patient to journalists and to her mother

Bulgaria is bound by the Convention; these principles inform how Health Act Arts 28–28c and the GDPR are applied.

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A teaching scale of breaches

Degree	Nature	Example
Zero	lawful sharing for care	the treating doctor sends results to the consultant (Code Art. 53)
First	careless, low identifiability	an unnamed case discussed in the lift
Second	unauthorised recipient or access without need	a staff member opens an acquaintance's record; information to a caller whose identity was not checked
Third	deliberate, public or large-scale	a recognisable photo on social media; records sold or leaked

Lack of intent does not remove the breach

Intent affects the type and extent of liability, not whether confidentiality was breached.

Teaching and conferences

Code Art. 54: anonymity must be **guaranteed** and identification by third parties excluded; otherwise the patient's consent is needed.

Illustrative teaching scale from the textbook (ch. 7), not statutory; assess identifiability, sensitivity, recipients, consequences and culpability separately

Digital confidentiality and the response to a breach

Risks

- shared logins, unlocked screens, printed lists
- personal messaging apps and e-mail for clinical images
- photos of wounds, screens, X-rays on personal phones
- external artificial-intelligence tools fed identifiable data
- "curiosity browsing" in the electronic record

> *Medical and non-medical specialists in medical and health establishments must create an electronic health record for every activity performed ..., sent to the National Health Information System with a qualified electronic signature.*
>
> — Health Act, Art. 28d(3a) (abridged)

After a breach

1. **Contain** — delete the post, recall the e-mail, lock the account.
2. **Report** at once through the internal channel (data protection officer).
3. **Assess** the data and the people affected.
4. **Notify** the Commission for Personal Data Protection within 72 hours of becoming aware, unless no risk is likely (GDPR Art. 33), and the patient if high risk (Art. 34).
5. **Learn** — change the process, not only blame the person.

Concealing a breach multiplies the harm.

One breach, four kinds of liability

Kind	Legal basis	Who decides
1 · Disciplinary	Labour Code, Arts 186–188: culpable breach of work duties, incl. duties under laws and internal rules (Art. 187(1)(10))	the employer, after hearing the employee (Arts 192–193)
2 · Administrative	Health Act, Art. 221 (patient rights — the establishment) and Art. 229 (general); GDPR Art. 83 and PDPA (controller, processor)	Art. 221: Medical Supervision Executive Agency (Art. 233a); Art. 229: Regional Health Inspectorate or Medical Supervision (Art. 235); Commission for Personal Data Protection
2a · Professional body	Professional Organisations Act, Arts 37(1)(1), 38–41; Code Arts 51–55	ethics committee of the regional college; decree by the chair of its board (Art. 40)
3 · Criminal	Penal Code, Art. 145(1)	the court, on the victim's complaint (Art. 161(1))
4 · Civil	Obligations and Contracts Act, Arts 45, 49, 52; PDPA Art. 39(2); GDPR Art. 82	the court, on the injured person's claim

Professional penalties do not exclude criminal, civil or disciplinary liability (Professional Organisations Act, Art. 42(1)); disciplinary penalties apply regardless of other liability (Labour Code, Art. 186).

Criminal and professional liability

> *A person who unlawfully discloses another's secret, dangerous to someone's good name, which has been entrusted to him or has become known to him in connection with his occupation, shall be punished by imprisonment of up to one year or a fine.*
>
> — Penal Code, Art. 145(1)

Prosecution only on the **victim's complaint** (Penal Code, Art. 161(1)). The offence is **intentional**: negligent acts are punishable only where the law expressly says so (Art. 11(4)). Not every disclosure is a crime: the secret must be capable of damaging someone's good name.

> *For violations under Art. 37(1)(1) [breach of the codes of professional ethics] the following penalties may be imposed: 1. reprimand; 2. a fine of one to five minimum monthly wages; 3. removal from the register of the college for 3 months to one year.*
>
> — Professional Organisations Act, Art. 38

Complaints are heard by the ethics committee of the regional college (Art. 39); a member draws up the act; the chair of the regional board issues the penalty decree (Art. 40); appeal under the Administrative Violations and Penalties Act (Art. 41).

Civil liability: compensation for the patient

> *Everyone is obliged to repair the harm he has culpably caused to another. In all cases of tort, fault is presumed until proven otherwise.*
>
> — Obligations and Contracts Act, Art. 45

> *A person who has assigned work to another is liable for the harm caused by him in or in connection with the performance of that work.*
>
> — Obligations and Contracts Act, Art. 49

> *Compensation for non-pecuniary harm is determined by the court on an equitable basis.*
>
> — Obligations and Contracts Act, Art. 52

What the patient must show

unlawful disclosure $\longrightarrow$ harm (pecuniary or non-pecuniary: shame, stigma, lost job) $\longrightarrow$ causation. Fault is presumed; the establishment answers with the employee.

Also: damages for unlawful processing of personal data (PDPA Art. 39(2); GDPR Art. 82).

Small-group work: cases 7–10

Task for each group

1. Identify the **information**, the **requester** and the **purpose**.
2. Name the legal ground — or state that there is none.
3. Propose the **least intrusive** action and the **minimum** disclosure.
4. Say what you would **record**.

No.	Case
7	The photograph in the group chat
8	Huntington's disease in the family
9	The patient who wants to kill his boss
10	The bruises of an elderly woman

6 minutes of work in parallel · then 2 minutes per group: report and key points.

Case 7 · The photograph in the group chat

The case

A junior doctor treats a child with a rare rash. Wanting a quick second opinion, she photographs the lesions on her personal phone and posts them, with the child's age, the admission date and the hospital, in a private messaging group of eighty clinicians. She omits the name. A member recognises the child and forwards the post to a dermatologist outside the group. The parents had agreed to clinical photography **for the record** only.

1. Which separate decisions were taken (photograph, store, post, forward)?
2. Why is omitting the name not anonymisation?
3. What must the doctor and the hospital do now?

Case 7 · The photograph in the group chat — key points

Law and liability

- Identifiable health data (Health Act Art. 27(1)) disclosed outside any lawful basis: breach of Art. 28c and Code Arts 51–52 (illustrative material); a consultation is lawful through the proper route (Code Arts 35–36, 53), not an open group.
- The parents' consent covered the record, not a messaging group.
- The hospital is the controller: internal report, risk assessment, notification within 72 hours of becoming aware unless no risk is likely (GDPR Art. 33), information to the parents if high risk (Art. 34).
- Possible: disciplinary, professional, civil liability.

Practice

- The purpose (a second opinion) was legitimate; the **route** was wrong: use the hospital's secure system or a formal consultation (Code Arts 35–36).
- Delete the post, ask members to delete copies, report — do not hide it.
- Explain to the parents honestly, without speculation; apologise.

Case 8 · Huntington's disease in the family

The case

Mr D., 51, has been treated for depression for seven years. He develops involuntary movements; genetic testing confirms Huntington's disease. He was adopted and knew nothing of his biological family. He and his wife decide that their three adult children must not be told: "the knowledge would be a burden — and what can they do about it anyway?" Each child has a 50% risk.

1. Does the result belong to Mr D. or to the family?
2. Do the parents have an ethical duty to tell the children?
3. May the doctor tell the children without consent?

Case 8 · Huntington's disease in the family — key points

Law

- Without his consent, disclosure would have to meet Health Act Art. 28(1)(2) (threat to the health or life of others) with prior notification (Art. 28(2)).
- The application of the "threat" ground is uncertain and fact-dependent: the children's interests concern reproductive and life planning and psychological health; seek genetic and legal advice.
- *Comparative:* **ABC v St George's** (2020) — a duty to balance arising from a close relationship with that relative; no breach established.

Ethics

- The children also have a **right not to know**; an unsolicited result may itself harm.
- The doctor's task: genetic counselling, exploring the parents' fears, offering help to tell the children, especially if they plan families.
- Keep the door open: the decision is reviewed as circumstances change (e.g. a child asks, or plans a pregnancy).

Case 9 · The patient who wants to kill his boss

The case

A psychiatrist sees a patient who has become convinced that his boss is persecuting him. The patient asks: "Can you keep a secret?" The psychiatrist replies: "Of course — everything you tell me will always stay confidential." The patient then says he is planning to kill his boss "when the time is right".

1. What was wrong with the psychiatrist's answer?
2. Is there a legal ground to disclose, and to whom?
3. What comes first: treatment, warning or both?

Case 9 · The patient who wants to kill his boss — key points

Law

- A serious threat to the life of an identifiable person: Health Act Art. 28(1)(2) permits disclosure, after the patient is notified (Art. 28(2)). If notifying him would itself create immediate danger, urgent protection needs separate legal assessment — the law states no exception.
- Recipients: the police and/or the person at risk; the minimum needed to protect.
- Separately: emergency psychiatric help requires a direct and immediate danger (Art. 153); compulsory placement follows the judicial procedure (Arts 155–156; Class 2).
- *Comparative:* **Tarasoff** (1976).

Ethics and practice

- The promise of **absolute** confidentiality was a mistake; correct it honestly.
- Assess the risk: plan, means, timing, history of violence, insight.
- Treatment and protection go together; record the assessment, the advice taken and the disclosure.

Case 10 · The bruises of an elderly woman

The case

A 75-year-old woman has bruises of different ages on her arms and back that suggest she is being hit. Her husband, her main carer, brings her to the appointments. When you see her alone, she confirms that he "sometimes loses his temper", but begs you not to say anything: she depends on him and is afraid of being sent to a care home. She has no cognitive impairment.

1. Whose decision is it?
2. When may you act against her wish?
3. What can be done without breaking her confidence?

Case 10 · The bruises of an elderly woman — key points

Law

- She is an adult with decision-making capacity: disclosure to the police or social services normally needs her **consent**, unless a specific statutory reporting duty applies.
- Art. 28(1)(2) concerns danger to **others**, not to her alone. If a child lives in the household, assess whether it needs protection (exposure to violence) — then the Child Protection Act, Art. 7, applies.
- Document the injuries carefully (dated, measured, photographed with consent) — the record may later protect her.

Ethics and practice

- Respect her autonomy without abandoning her: a safety plan, information on protective measures and support services, contact numbers, follow-up appointments seen alone.
- Re-assess capacity and coercion over time — dependence can make "consent" to abuse less than free.
- If her life is in immediate danger: emergency care and a call for help; take legal advice on the basis for further steps.

Summary

Ethics

1. Confidentiality protects autonomy, trust and the person from harm — it is a strong **presumption**, not an absolute rule.
2. Sharing for care is justified by **purpose, relevance and access**, not by working in the same hospital.
3. Every disclosure is **minimum**, directed to the right recipient, and documented.

Bulgarian law

1. Health information: Health Act Art. 27; prohibition: Art. 28c; protection: Art. 28(3).
2. Disclosure needs a lawful basis: the patient's authorisation, a ground in Art. 28(1) (danger to others: prior notification, Art. 28(2)) or another specific provision.
3. Patient, authorised person, heirs: Art. 28b. Child at risk: Child Protection Act Art. 7. Liability: four kinds.

Before you share: What information? Who asks? On what basis? How much? Through which channel? Is it recorded?

Thank you for your attention!